

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the supply/administration of Clobetasone or Betamethasone cream for the treatment of Eczema or Dermatitis

Summary of NICE / NICE CKS Guidelines for Eczema / Dermatitis

Overview

- **Eczema (dermatitis)** is a **chronic, relapsing inflammatory skin condition** characterised by **itch, dryness, and inflammation**.
- Common types include:
 - o **Atopic eczema** (most common, especially in children)
 - o **Contact dermatitis** (irritant or allergic)
 - o **Seborrhoeic dermatitis**
 - o **Discoid and varicose eczema**

Assessment

- Assess **severity, distribution, triggers, and impact on quality of life**.
- Look for signs of **infection** (weeping, crusting, worsening inflammation).
- Identify **exacerbating factors** (e.g. allergens, soaps, clothing, stress).
- Consider **patch testing** for suspected allergic contact dermatitis.

General Management Principles

1. Emollients (First-Line for All Types)

- Use **liberally and frequently**, even when skin is clear.
- Types: ointments (greasier, for dry skin), creams/lotions (less greasy, for daytime use).
- Apply **at least twice daily** and after bathing.

2. Topical Corticosteroids

- Used **during flare-ups** for inflammation.

- **Potency depends on site and severity:**
 - o Mild: e.g. **hydrocortisone 1%**
 - o Moderate: e.g. **clobetasone butyrate**
 - o Potent: e.g. **betamethasone valerate 0.1%**
- Use **lowest potency for shortest duration** possible.
- **Step up/down** based on response and body area.

Apply **once or twice daily**, after emollients (leave 20–30 mins between).

3. Other Topical Treatments

- **Topical calcineurin inhibitors** (e.g. tacrolimus, pimecrolimus):
 - o For sensitive areas (e.g. face, eyelids) or steroid-sparing in moderate-severe eczema.
 - o Specialist initiation.

Infection Management

- Signs of infection: weeping, crusting, sudden worsening, fever.
- **Topical fusidic acid** for localised infection (short-term only).
- **Oral antibiotics** (e.g. flucloxacillin) for widespread/systemic infection.
- **Swab if recurrent or treatment failure**; consider MRSA.

Allergy and Irritant Contact Dermatitis

- Identify and avoid the **trigger** (e.g. soaps, chemicals, metals).
- Use **protective gloves** and barrier creams where applicable.
- Refer for **patch testing** if allergy suspected.

When to Refer

- Diagnostic uncertainty
- Severe or resistant eczema
- Recurrent infections
- Psychosocial impact
- Suspected allergic contact dermatitis needing testing
- Consider referral for **phototherapy or systemic treatments** (e.g. ciclosporin, dupilumab)



Patient Education and Support

- Demonstrate how to apply treatments.
- Discuss **flare-up action plans**.
- Encourage adherence to emollients even when well-controlled.

Patient Group Direction

for the supply/administration of

Clobetasone butyrate

for the treatment of

Eczema or Dermatitis

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Clobetasone butyrate is indicated for the short-term treatment of mild to moderate eczema or dermatitis in adults and children aged 12 years and over where a topical corticosteroid is appropriate.
Inclusion criteria	- Individuals aged 12 years and over. - Diagnosis of mild to moderate eczema or dermatitis. - Informed consent obtained. - No signs of secondary infection.
Exclusion criteria	- Hypersensitivity to clobetasone or any component of the cream or ointment. - Untreated skin infections (fungal, bacterial, viral). - Use on broken, ulcerated, or weeping skin. - Long-term or repeated use without review. - Pregnancy or breastfeeding unless deemed appropriate by prescriber.
Cautions	- Avoid contact with eyes and mucous membranes. - Do not use under occlusion unless advised by a prescriber. - Limit treatment duration and area to avoid steroid overuse. - Advise on appropriate emollient use alongside corticosteroid. - For external use only. This and all medication should be kept out of the reach of children. In case of accidental ingestion, professional assistance should be sought or a national poison control centre contacted immediately. - Product contains paraffin and is a flammability risk. Instruct patients not to smoke or go near naked flames due to the risk of severe burns. Fabric (clothing, bedding, dressings etc.) that has been in contact with this product burns more easily and is a serious fire hazard. Washing clothing and bedding may reduce product build-up but not totally remove it. - Prolonged application to the face is undesirable as this area is more susceptible to atrophic changes. - Visual disturbance may be reported with systemic and topical corticosteroid use. If a patient presents with symptoms such as blurred vision or other visual disturbances, the patient should be considered for referral to an ophthalmologist for evaluation of possible causes.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	0.05% w/w clobetasone butyrate cream or ointment
Legal category	POM

Storage	Store below 25°C.
Route/method of administration	Topical application to affected area once or twice daily. Apply thinly and gently rub in using only enough to cover the affected area. Creams are especially appropriate for moist or weeping surfaces. Ointments are especially appropriate for dry, lichenified or scaly lesions. Once improvement occurs, reduce the frequency of application or change the treatment to a less potent preparation. Therapy with topical corticosteroids should be gradually discontinued once control is achieved and an emollient continued as maintenance therapy. Allow adequate time for absorption after each application before applying an emollient.
Dose and frequency	Apply a thin layer to the affected area once or twice daily. Continuous daily treatment for longer than four weeks is not recommended.
Quantity to be administered and/or supplied	One 30 g tube per treatment course.
Maximum or minimum treatment period	Use for up to 7 days initially; review if extended use is required.
Adverse effects	Skin thinning, burning, itching, irritation. Rare: allergic dermatitis, infection exacerbation. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the

	patient was 17 years when treatment was finished, keep records until the 26 th birthday)
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Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 3–4 weeks, or they become systemically very unwell. Provide instructions on the correct use of emollients, with clear demonstrations where appropriate. Address any concerns regarding adverse effects of topical corticosteroids. Encourage the frequent and liberal use of emollients, even during periods where the skin is clear. Regular use of emollient therapy has been shown to have both short and long-term steroid sparing effects in mild to moderate atopic eczema/dermatitis. Advise that where possible, they should avoid trigger factors known to exacerbate eczema/dermatitis, such as certain clothing, soaps or detergents, animals and heat. Advise that patients should try and keep their nails short and avoid scratching and simply rub the area with their fingers to alleviate itch. Inform that the condition may temporarily affect skin pigmentation (causing the skin to become lighter or darker).

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines [BNF \(British National Formulary\) | NICE](#)
- Summary of Product Characteristics

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature

Valid from date		Expiry date	
1/11/25		31/10/26	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026
Name of professional group signatory	Job title & name of organisation	Signature	Date
Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/2025
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/2025

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1 st November 2025

Patient Group Direction

for the supply/administration of

Betamethasone valerate

for the treatment of

Eczema or Dermatitis

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Betamethasone valerate is indicated for the short-term treatment of mild to moderate eczema or dermatitis in adults and children aged 12 years and over where a topical corticosteroid is appropriate.
Inclusion criteria	- Individuals aged 12 years and over. - Diagnosis of mild to moderate eczema or dermatitis. - Informed consent obtained. - No signs of secondary infection.
Exclusion criteria	- Hypersensitivity to betamethasone or any component of the cream or ointment. - Untreated skin infections (fungal, bacterial, viral). - Use on broken, ulcerated, or weeping skin. - Long-term or repeated use without review. - Pregnancy or breastfeeding unless deemed appropriate by prescriber.
Cautions	- Avoid contact with eyes and mucous membranes. - Do not use under occlusion unless advised by a prescriber. - Limit treatment duration and area to avoid steroid overuse. - Advise on appropriate emollient use alongside corticosteroid. - Product contains paraffin and is a flammability risk. Instruct patients not to smoke or go near naked flames due to the risk of severe burns. Fabric (clothing, bedding, dressings etc.) that has been in contact with this product burns more easily and is a serious fire hazard. Washing clothing and bedding may reduce product build-up but not totally remove it. - Prolonged application to the face is undesirable as this area is more susceptible to atrophic changes. - Visual disturbance may be reported with systemic and topical corticosteroid use. If a patient presents with symptoms such as blurred vision or other visual disturbances, the patient should be considered for referral to an ophthalmologist for evaluation of possible causes.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	0.1% w/w betamethasone valerate cream or ointment.
Legal category	POM
Storage	Store below 25°C.
Route/method of administration	Topical application to affected area once or twice daily. Apply thinly and gently rub in using only enough to cover the affected area.

	Creams are especially appropriate for moist or weeping surfaces. Ointments are especially appropriate for dry, lichenified or scaly lesions. Once improvement occurs, reduce the frequency of application or change the treatment to a less potent preparation. Therapy with topical corticosteroids should be gradually discontinued once control is achieved and an emollient continued as maintenance therapy. Allow adequate time for absorption after each application before applying an emollient.
Dose and frequency	Apply a thin layer to the affected area once or twice daily.
Quantity to be administered and/or supplied	One 30 g tube per treatment course.
Maximum or minimum treatment period	Use for up to 7 days initially; review if extended use is required.
Adverse effects	Skin thinning, burning, itching, irritation, telangiectasia. Rare: systemic absorption with prolonged use. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

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